

Chapter 65 — Adolescent and Transitional Urology — Numbers, Gold Standards and Traps

IF YOU READ NOTHING ELSE

1. **Start transition ~age 12** (American Academy of Pediatrics, AAP); preparation 12–14
2. **Transfer 18–21 → complete 18–23**; latest completion commonly 26
3. **Three Ps**: Preparation, Plan, **named** Provider of the future
4. **Paediatric team initiates** transition, not the adult team
5. **Transfer ≠ transition**: complete = integration confirmed + feedback
6. **Active transition → fewer lost to follow-up**
7. **Outcome metric = stability**, not improvement (renal function, glycosylated haemoglobin)
8. **No transition during crisis**; exception **pregnancy** → co-management
9. **Emerging adults 18–25** consent for themselves unless capacity lacking
10. **Spina bifida (SB)**: normal intelligence; ↓ **working memory + executive function**

WHAT CONSTITUTES ADOLESCENT UROLOGY?

- **Adult reconstructive urologist**: operatively capable; **underequipped for whole patient**
- **Aim**: seamless lifelong care; adolescent urology = **bridge**

DEFINITION OF TRANSITION

- **Got Transition**: child/family-centred → **adult/patient-centred** care
- **Three parts**: planning → transfer → integration

WHY IS TRANSITION NEEDED?

- **AHRQ** (Agency for Healthcare Research and Quality): **"no significant difference"**, but metrics inadequate
- **Avoid "normal"**: patient = **own baseline**

WHO NEEDS TRANSITION?

- **1. Complex congenital** (exstrophy, posterior urethral valves, complex hypospadias, SB) → **lifelong**
- **2. Quiescent** (vesicoureteric reflux, VUR) → **may be discharged**, open return
- **3. Cured, treatment sequelae** (pelvic rhabdomyosarcoma) → late-effects surveillance
- **4. Beyond paediatric scope** → fertility, pregnancy

WHAT HAPPENS WHEN TRANSITION FAILS?

- **Failure** → non-adherence, ↑ emergency department visits, admissions, morbidity, mortality

TRANSITION AS A PROCESS

- **Six core elements**: policy 12–14 → tracking/readiness/ planning 14–18 → transfer 18–21 → complete 18–23
- **Readiness tool**: TRAQ (Transition Readiness Assessment Questionnaire)
- **Top preparation element**: how/where to find **valid information**
- **Private time alone** → sex, sexual function, sexual debut
- **Programmes**: Good 2 Go (Toronto), Ready Steady Go (UK), Got Transition (US)
- **Best programme** = the one that fits the unit
- **Age 26** = US limit on parent's insurance
- **No independent living** → plan medical custodianship

SYSTEMS AND MODELS OF CARE

- **Routes**: transition (1) + new adolescent problem (3) **ideal**; no-transition referral (2) suboptimal; **emergency after discharge (4) worst**
- **Lifelong paediatric urologist** → suits limited life expectancy; fails when patient outlives provider

- **Integrated paediatric + adult dept** → joint clinics; barriers: cost, geography, privileging
- **Direct transfer, no plan** → invisible until emergency
- **Multidisciplinary team (MDT) backbone**: nursing
- **Most complex MDT needs**: neurological disease (SB)

DEVELOPMENTAL CONSIDERATIONS

- **Genital surgery windows**: < 6 months (both); 2–3 y (theory) vs 4th–5th y (AAP 1975)
- **Stages**: bonding year 1; individuation 18–30 mo; phallic-oedipal 3–6 y
- **Augmentation cystoplasty**: child helps catheterise + wants to be dry
- **Exstrophy closure** (non-elective) → as soon as safe
- **Self-management components (4)**: adherence, responsibility, self-care skills, knowledge
- **Medical post-traumatic stress disorder**: no adult congenital urology data; cognitive-behavioural therapy mitigates (cardiac data)

THE PRACTICALITIES: SETTING UP A SERVICE

- **Training pathway**: none defined; usually paediatric + reconstructive
- **Core MDT**: paediatric + transitional urologist, nurses, coordinator, nephrologist, psychologist
- **Biggest adult-provider barrier**: unfamiliarity with the condition
- **Paediatric provider**: "Am I the best person?"

CLASSIC TRAPS

- **Trap**: AHRQ shows transition futile — **Actually**: poor metrics; stability = success
- **Trap**: transfer = transition — **Actually**: transfer one step; complete 18–23
- **Trap**: all need lifelong follow-up — **Actually**: quiescent (VUR) may be discharged
- **Trap**: MDT need = lifelong need — **Actually**: exstrophy lifelong, urologist-led
- **Trap**: new adolescent problem suboptimal — **Actually**: route 3 ideal
- **Trap**: lifelong paediatric urologist best — **Actually**: fails as patient outlives provider
- **Trap**: elective timing follows theory — **Actually**: school, parental anxiety, surgeon
- **Trap**: weak processing → parent consents — **Actually**: capacity present; support consent
- **Trap**: confidentiality reviewed once — **Actually**: with family, then alone
- **Trap**: adult team starts transition — **Actually**: paediatric team initiates